The Contextual Matrix: How Institutions, Capital, and Culture Dictate Diagnosis

Introduction: The Diagnostic Montage

Imagine these three cases: in a manicured Connecticut living room, a high school junior ticks boxes on a TikTok "ADHD symptom checklist," lobbying his private psychiatrist into a diagnosis. He soon secures a prescription and a fifty-percent SAT time extension—instruments likely to determine his elite university admission. In a different suburb, a fifteen-year-old mourning his parents' divorce plays an immersive role-playing game six hours daily to manage panic. His school counselor flags him for "Internet Gaming Disorder," renaming grief as pathology. Meanwhile, a thirty-year-old woman weeps in relief; a clinician has finally diagnosed her ADHD, properly identifying a lifetime of chronic burnout that her parents dismissed as female hysteria.

Though superficially similar, these encounters obey fundamentally different logics. The question "whether mental illness is over-diagnosed or better recognized"—presents a structurally incomplete binary: the answer depends entirely on context. Mental illness is aggressively over-diagnosed where a medical label is converted into competitive capital, deployed to individualize structural failure, or used to validate a moral panic. Conversely, it is genuinely better recognized where marginalized demographics dismantle the diagnostic manual's historical biases, and where clinical psychology distinguishes pathology from self-directed therapeutic regulation. The DSM is less an objective biological ledger than a sociological mirror: it reflects the interests, anxieties, and blind spots of the institutions utilizing it.

Contexts of Over-Diagnosis: The Diagnostic Label as Structural Alibi

The first mechanism of over-diagnosis operates from above, when a state apparatus or institution develops an interest in locating social dysfunction inside the individual rather than the surrounding environment. The case of "Gaming Disorder" is exemplary. When the World Health Organization moved to include Gaming Disorder in the ICD-11, a multi-author open debate paper, led by media theorist Espen Aarseth, argued that the psychiatric establishment had simply ported the diagnostic grammar of substance addiction over to frequent gaming, without doing the basic cultural work of asking what such gaming was for (Aarseth et al., 2017). Frequent gaming is often a functional response to environments that deny young people autonomy, physical community, and unstructured time. The teenagers spending six hours a day in an immersive RPG are not addicts; they are citizens of collapsed third places. The diagnosis functions as a moral-panic technology, transforming a structural condition—the hollowing-out of civic space—into a private pathology of the child's reward system. The state and school can then withdraw from the problem, reassured by clinicians that the problem is neurological.

The same logic operates regarding Major Depressive Disorder (MDD). As clinical psychologist David Smail argued, the steady expansion of the depression category functions less as a genuine clinical breakthrough than as a "magical" solution to what are, at root, material social problems (Smail, 1993). The depressive symptom is the predictable manifestation of being powerless—poor, badly housed, precariously employed, or socially isolated. The diagnosis of "depression" individualizes that output, routing the patient toward a private pharmacological solution while leaving the social conditions of their suffering untouched (Smail, 1993). To diagnose the symptom is to absolve the structure.

Contexts of Over-Diagnosis: The Diagnostic Label as Systemic Capital

If the state leverages the diagnostic label to absolve itself of structural responsibility from above, individuals trapped within hyper-competitive structures quickly learn to weaponize the label from below.

When institutions become rigid, metrics-driven, and unresponsive to standard human friction, citizens learn that a medical diagnosis is the only administrative key that forces the system to yield. This dynamic is best understood through Nikolas Rose and Carlos Novas's concept of "biological citizenship"—the phenomenon where individuals leverage somatic or psychological classifications to make legal or administrative claims on institutions (Rose & Novas, 2004). Consider a university student assigned to a cramped dormitory fundamentally unconducive to study. Rather than engaging in a futile battle with an inflexible housing bureaucracy, she pays a $150 fee to an online telehealth service. After a fifteen-minute consultation, she receives a diagnosis of Adjustment Disorder and an Emotional Support Animal (ESA) certification for her cat. Armed with these psychiatric documentations, she legally bypasses the university's housing policies and is reassigned to a coveted, single-occupancy studio apartment. The psychological label becomes the only language the rigid bureaucracy is legally forced to respect; the diagnosis is utilized to extract spatial resources.

Nowhere is this capitalization of the psychiatric label more aggressively optimized than in the hyper-meritocratic arena of elite education. To understand why Attention Deficit Hyperactivity Disorder (ADHD) and Specific Learning Disorders (SLD) have become tradable instruments, it is necessary to follow the logic of what Peter Conrad termed the "medicalization of deviance" (Conrad, 2007). In the modern educational arms race, however, it is no longer just deviance being medicalized; it is competitive friction. Take

the case of a high-achieving prep school sophomore with a B+ average, facing the cold reality that her grades will not clear the academic threshold for an Ivy League admission. Her parents hire a private neuropsychologist for a $5,000 evaluation. The clinician identifies a slight discrepancy in speed of thought and codes it as a Specific Learning Disorder. This label is immediately converted into a 100% time extension on Advanced Placement exams and the SAT, effectively neutralizing the tests' time pressure. Under Pierre Bourdieu's framework of "cultural reproduction," this is a flawless conversion of capital: the medical label allows affluent families to convert economic capital (the cost of the private assessment) into institutional capital (the test accommodations) (Bourdieu, 1986).

Taken together, what these contexts for the diagnosis as structural alibi or systemic capital reveal is that the DSM functions as a sociological contract—a mechanism for negotiating resources and responsibility in late capitalism. Whether governments avoid public housing via MDD diagnoses, or teenagers bypass SAT limits via SLD classifications, the label distributes responsibility, ensuring the surrounding social and economic arrangements remain structurally intact.

Contexts of Better Recognition: Dismantling the Demographical Gaze

If over-diagnosis is a story about extending the medical category to serve institutional or personal interests, the phenomenon the diagnostic manuals are currently undergoing among women is, structurally, the opposite movement: a long-overdue contraction of a medical blind spot. This correction is most visible in ADHD and Autism Spectrum Disorder, the diagnostic criteria of which were historically built almost entirely from observing disruptive boys. The DSM criteria for ADHD lean heavily on hyperactivity

and visible classroom disruption; the prototypical case is a boy who cannot sit still. Research has demonstrated that women and girls with ADHD, by contrast, present an "internalizing" symptom profile: chronic disorganization, inattention, emotional dysregulation, and a quieter restlessness that rarely gets a child sent to the principal's office (Quinn & Madhoo, 2014). The diagnostic gaze was constructed to see a single, gendered silhouette, and the suffering of the women who did not fit it went structurally uncounted.

Autism tells the same story with an even longer lag. The "first descriptions" of the condition by Kanner and Asperger were of boys, and the diagnostic instruments subsequently built from those descriptions—the ADOS and ADI-R standardized assessments, and the DSM-5 criteria themselves—preserve that male prototype in their architecture (Kanner, 1943; Asperger, 1944). Subsequent research has shown that the female autism phenotype is qualitatively different from the male one: autistic women present with fewer restricted and repetitive behaviors, more social mimicry, more active camouflaging of autistic traits, and a different profile of special interests (Hull, Petrides, & Mandy, 2020). The camouflaging hypothesis they elaborate—that autistic women have historically learned to mask their traits to pass as neurotypical—has become a generative framework. Other scientists have concluded that existing instruments consistently underestimate the prevalence of autism in females, forcing the field to recalibrate accordingly (Cook, Hull, & Mandy, 2024).

The thirty-year-old woman in the introduction was misdiagnosed for decades because the diagnostic system was previously incapable of recognizing her presentation. Interviews with late-diagnosed autistic women have made this systematic misrecognition visible: a long history of being labeled "anxious," "too sensitive," or "weird" rather than

autistic (Bargiela, Steward & Mandy, 2016). The recent surge in female diagnoses is not inflation; it is the contested work of feminist epistemology forcing the diagnostic manual to contract the space of "normal" female behavior that swallowed the autistic and inattentive. The label is being bent to fit the patient.

Contexts of Better Recognition: Validating Self-Directed Therapeutics

The fifteen-year-old gamer in the introduction can be read in a way that the contemporary clinical literature is only beginning to make room for: he is running a self-directed therapeutic protocol. His immersive role-playing game provides three things his environment couldn't: a stable narrative with predictable moral consequences, perceived control over difficult emotions, and a low-stakes social world rewarding competence. For him, game-playing is not a psychiatric compulsion; it is a regulatory choice.

Granic, Lobel, and Engels's 2014 article in *American Psychologist*—"The Benefits of Playing Video Games"—is the most cited articulation of this reframing. Against moral-panic discourses, they argue video games are best understood as complex, cognitively demanding environments through which players practice emotion regulation, resilience, and social coordination (Granic, Lobel, & Engels, 2014). Their review of experimental literature correlates moderate-to-intensive gameplay with improved mood-regulation, faster recovery from negative affect, and enhanced problem-solving. The developmental default is not pathology.

This regulatory logic runs through the young adult running ten kilometers after a panic attack, or the office worker using a paced-breathing app to manage an unnamed anxiety disorder. Patients often design neuro-regulatory interventions addressing the actual

texture of their distress without professional guidance. The self-directed-therapeutics account demands a recognition where the clinician's first move is not to determine the DSM category, but to ask what regulatory work the behavior is doing. The clinical task is to validate what is already working and help refine it, rather than substitute a new diagnostic label for the patient's own efforts towards bringing order to life.

Conclusion: The Diagnostic Mirror, and Toward a Politics of Recognition

Realities described in the previous four sections are instances of a single structural fact. The DSM is not an objective biological ledger; it is a sociological mirror. The categories within it—ADHD, SLD, Gaming Disorder, MDD, Autism—are the categories that the writing institutions could justify at the moment of writing, to the constituencies they were answerable to, under the social conditions that prevailed. Thus, the binary of the question "whether mental illness is over-diagnosed or better recognized" collapses. The question in any clinical encounter is not the abstract accuracy of the label, but who is being asked to bear its cost, and what institutional work it is being made to do.

If the DSM is a sociological contract, the ultimate question is who the diagnostic system is designed to serve. Moving forward, several constructive acts could be done. First, clinical and institutional frameworks must decouple, where possible, the institutional benefits that follow a psychiatric label from the label itself. The high school junior in the introduction did not need an ADHD diagnosis in order to need extended time on the SAT; the test was the problem, not his brain. Second, in clinical contexts, a routine structural-context intake should precede, not follow, the diagnostic act. Third, the diagnostic gaze must continue to decentralize—adopting diagnostic tools that can detect camouflaged presentations of autism and ADHD in women, validated on the populations they claim to

diagnose. Lastly, clinicians should approach the patient's self-directed regulatory strategies as evidence, not noise. The teenager's gaming, the worker's running, the patient's paced breathing are not waiting to be pathologized; they are, often, the most informative datum in the room.

None of this is utopian. Each of these moves is being implemented somewhere already. What is required, if the diagnostic mirror is to reflect something other than the interests of the institutions that hold it, is a sustained willingness to design the clinical encounter around the patient—and to design the institution around the clinical encounter. Until the question of "who the diagnostic system is for" is asked and answered with commitment, no revision of the DSM will, on its own, be enough.

References

Aarseth, E., Bean, A. M., Boonen, H., Colder Carras, M., Coulson, M., Das, D., Deleuze, J., Dunkels, E., Edman, J., Ferguson, C. J., Haagsma, M. C., Helmersson Bergmark, K., Hussain, Z., Jansz, J., Kardefelt-Winther, D., Kutner, L., Markey, P., Nielsen, R. K. L., Prause, N., … Van Rooij, A. J. (2017). Scholars' open debate paper on the World Health Organization ICD-11 Gaming Disorder proposal. *Journal of Behavioral Addictions*, 6(3), 267–270. https://doi.org/10.1556/2006.5.2016.088

Asperger, H. (1944). Die "Autistischen Psychopathen" im Kindesalter. *Archiv für Psychiatrie und Nervenkrankheiten*, 117, 76–136.

Bargiela, S., Steward, R., & Mandy, W. (2016). The experiences of late-diagnosed women with autism spectrum conditions: An investigation of the female autism phenotype. *Journal of Autism and Developmental Disorders*, 46(10), 3281–3294. https://doi.org/10.1007/s10803-016-2872-8

Bourdieu, P. (1986). The forms of capital. In J. Richardson (Ed.), *Handbook of theory and research for the sociology of education* (pp. 241–258). Greenwood.

Conrad, P. (2007). *The medicalization of society: On the transformation of human conditions into treatable disorders*. Johns Hopkins University Press.

Cook, J., Hull, L., & Mandy, W. (2024). Improving diagnostic procedures in autism for girls and women: A narrative review. *Neuropsychiatric Disease and Treatment*, 20, 505–514. https://doi.org/10.2147/ndt.s372723

Granic, I., Lobel, A., & Engels, R. C. M. E. (2014). The benefits of playing video games. *American Psychologist*, 69(1), 66–78. https://doi.org/10.1037/a0034857

Hull, L., Petrides, K. V., & Mandy, W. (2020). The female autism phenotype and camouflaging: A narrative review. *Review Journal of Autism and Developmental Disorders*, 7(3), 306–317. https://doi.org/10.1007/s40489-020-00197-9

Kanner, L. (1943). Autistic disturbances of affective contact. *Nervous Child*, 2, 217–250. https://neurodiversity.com/library_kanner_1943.pdf

Quinn, P. O., & Madhoo, M. (2014). A review of attention-deficit/hyperactivity disorder in women and girls: uncovering this hidden diagnosis. *The primary care companion for CNS disorders*, 16(3), PCC.13r01596. https://doi.org/10.4088/PCC.13r01596

Rose, N., & Novas, C. (2004). Biological citizenship. In A. Ong & S. Collier (Eds.), *Global assemblages: Technology, politics, and ethics as anthropological problems* (pp. 439–463). Blackwell. https://doi.org/10.1002/9780470696569.ch23

Smail, D. (1993). *The origins of unhappiness: A new synthesis*. Constable.